

Hypertension: Blood Pressure and Electrolyte Monitoring

This is a synthetic, simplified reference document created for a portfolio software project. It is not real clinical guidance and must not be used for patient care.

Patients on antihypertensive therapy, particularly ACE inhibitors, angiotensin receptor blockers, and potassium-sparing diuretics such as spironolactone, should have serum potassium and creatinine checked within one to two weeks of starting or up-titrating therapy, and periodically thereafter.

A serum potassium reference range of 3.5-5.1 mmol/L is considered normal. Values above 5.5 mmol/L represent hyperkalemia and should prompt medication review; values above 6.5 mmol/L are a critical result requiring urgent evaluation, including consideration of cardiac monitoring, given the risk of life-threatening arrhythmia.

Combining an ACE inhibitor with a potassium supplement or a second potassium-sparing agent increases hyperkalemia risk and should be monitored closely rather than avoided outright when clinically indicated, since these combinations are sometimes necessary for adequate blood pressure or heart failure management.

Sodium should also be monitored periodically, particularly in older adults or those on diuretics, as both hyponatremia and hypernatremia can present subtly with confusion, fatigue, or falls before becoming severe.